Auscultation

Fetal heart rates can be identified by Doppler ultrasound by 12 – 24 weeks and by fetoscope at 18– 20 weeks.

Investigations

- Urine test for confirmation of pregnancy, albumin and sugar
- Blood test for
  1. grouping
  2. Rh-typing
  3. Blood sugar
  4. Haemoglobin
  5. HIV Antigen.
  6. VDRL
  7. HbsAg.

Antenatal Advice

Nutrition

- Weight gain - the recommended weight gain in a normal pregnancy is 11.5 to 16 kg. Failure to weight gain may lead to IUGR, Low birth weight babies and poor perinatal outcome. Excessive weight gain may be due to fluid retention pre-eclampsia.
- Calories & proteins – The pregnant women require 300 kcal/day protein promotes growth of the fetus, placenta, uterus, breast, red cells and production of milk. During pregnancy 1 g of protein is deposited half to the fetus and half to mothers.
- Fats – Fats are important sources of energy. Phospholipids lower the surface tension in the lungs of the newborn.
- Iron – Recommended iron intake is 30 – 60 mg of elemental iron per day. During pregnancy iron stores are depleted, supplementary iron is needed for both mother and fetus.
- Calcium – Recommended calcium intake is 1200 mg per day.
- Vitamins – Folic acid is required for the formation of heme. Deficiency of folic acid may cause megaloblastic anaemia and neural tube defect in fetus.

Life Style Modification

- Exercise– It is not necessary for a pregnant mother to limit her exercise but restriction may require in Placenta previa, Cervical incompetence, Pregnancy induced hypertension, Premature labour and multiple gestation.
- Travel – No harmful effects have been identified. A pregnant woman should move around every 2 hours to prevent venous stasis and thrombus formation.
- Bowel habit in pregnancy due to progesterone induced GI smooth muscle relaxes lead to increased transit time. Late in pregnancy compression on the bowel by the presenting part may cause constipation. Women may avoid constipation by liberal fluid intake, exercise and stool softner, bulking agents & mild laxatives.
- Nausea & vomiting are common in the first and second trimesters, usually in the morning. Small frequent meals and avoidance of strong odours.
- Sexual intercourse does no harm but should be avoided if there is pregnancy complication such as placenta praevia, rupture of membrane, preterm labour.
- Smoking– women who smoke often have smaller infants with increased perinatal morbidity. Mothers are encouraged to quit smoking completely during pregnancy.
- Alcohol should not be consumed during pregnancy. The fetal abnormalities associated with drinking (fetal alcoholic syndrome) include craniofacial defects, limb and CVS defects, prenatal and postnatal growth restriction and mental retardation.
- Caffeine has no increase in teratogenic or reproductive risk.

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